- Sclerotherapy - injecting a chemical solution into the haemorrhoid to shrink it.

Surgical management

- **Rubber band ligation**. placing one or two tiny rubber bands around the base of haemorrhoid to cut off its circulation.
- Haemorrhoidectomy
- **Haemorrhoids stapling** - Surgical staples block the blood flow to haemorrhoids

Nursing management

- Educate to take plenty of water.
- Care of drainage tubes

Complications

- Anemia.
- Strangulated hemorrhoid.
- Infection
- Anal fistula

Prevention

- Eat high fibre foods.
- Drink plenty of fluids.
- Don't strain when trying to pass a stool
- Defecate as soon as the feeling of urge occurs
- Exercise.
- Avoid long periods of sitting Obesity



Nearly three out of four adults will have hemorrhoids

2.15 Renal Failure

Kidneys are two bean-shaped organs located in lower back. It excretes wastes and extra fluids from the body and producing and balancing chemicals that are necessary for body to function

Types

Acute real failure(ARF)

- Sudden onset
- Rapid reduction in urine output-usually irreversible

Chronic renal failure(CRF)

- Progressive
- Not reversible

Acute Renal Failure

Definition

Acute Renal Failure –is a sudden and almost complete loss of kidney function caused by failure of the renal circulation or by glomerular or tubular dysfunction

Causes of ARF

- **Pre-renal** (60-70 %)
- Volume Depletion
- Hemorrhage
- Renal losses
- Sepsis
- Cardiac failure
- Anaphylaxis

Intra renal (5-10%)

- Pigment Nephropathy
- Myoglobinuria
- Hemoglobinuria
- Nephrotoxic agents

Post-renal (20-40% cases)

- Urinary tract obstruction
- Calculi
- Tumors in lower urinary tract

People in risk

- Being hospitalized especially for a serious condition that requires intensive care
- Advanced age

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